

B. ASSESS, CLASSIFY AND TREAT THE SICK INFANT AGE 0 TO 2 MONTHS (0 TO 59 DAYS)

EVALUATE

QUICKLY EXAMINE ALL NEWBORNS WHO ARE WAITING

I. ASK THE MOTHER ABOUT THE CHILD'S PROBLEMS

Determine whether this is a first or follow-up visit for this problem.

- If this is a follow-up visit, follow the follow-up instructions.
- If this is your first visit, assess your baby's condition as follows:

CLASSIFY

TO CLASSIFY THE DISEASE, USE ALL THE FRAMES THAT CORRESPOND TO THE SYMPTOMS AND PROBLEMS OF THE

IDENTIFY TREATMENT

II. Look for a possible serious bacterial infection, very serious illness, pneumonia or local bacterial infection.

		SIGNS	CLASSIFY	TREATMENT
ASK Is the infant having difficulty feeding? Has the baby had convulsions (spasms)?	OBSERVE, LISTEN AND FEEL • See if the infant is currently convulsing • Counting breathing movements per minute, • Recount if the number is $\geq 60/\text{mn}$ • Listen to an expiratory whine • Look for a marked subcutaneous tug • Observe the infant's movements; if the infant is asleep, ask the mother to wake him up: - Does the infant move on own? If the infant does not don't move it, stimulate it gently. - Does an infant move only when stimulated? - it not move at ? • Taking an axillary temperature • Observe the navel: is it red or oozing pus? • Observe for pus draining from eyes • Look for skin pustules	One of the following signs • Unable to eat or not eat well or • Convulsions or • Rapid breathing (60 breaths per minute or more in infants under seven days old) or • Marked subcostal pull or • Expiratory whine or • Fever (38°C *or higher), or • Hypothermia (less than 35.5°C *) or • Moves only when stimulated or not at all, or • Rapid breathing (60 breaths per minute or more in infants aged 7 to 59 days)	POSSIBLE SEVERE BACTERIAL INFECTION OR VERY SEVERE MALA-DIEM	➤ If child is currently convulsing, administer anticonvulsants ➤ Administer the first dose antibiotic intramuscularly. ➤ Treat avoid hypoglycemia. ➤ Show the mother how to keep the infant warm during transfer in hospital. ➤ Refer URGENTLY hospital Or If referral is refused or not possible, treat at the health center referral is possible. (see table on page) ➤ Give oral amoxicillin for 7 days. ➤ Advise mother to provide home care. ➤ Follow-up in 3 days.
		• Umbilicus red or oozing pus <u>or</u> • Skin pustules <u>or</u> • Pus in the eyes	PNEUMONIE	➤ Give oral amoxicillin for 5 days. ➤ Teach the mother to treat local infections at home. ➤ Advise the mother to care for the infant at home. ➤ Follow-up in 2 days.
		No signs of bacterial infection or severe disease	LOCAL BACTERIAL INFECTION	➤ Treat for all other problems. ➤ Explain to the mother how to care for her baby at home.
			VERY SERIOUS ILLNESS OR BACTERIAL INFECTION IMPROBABLE	

* These thresholds are based on axillary temperature. Rectal temperature thresholds are about 0.5°C higher.

** If transfer is not possible, see *Managing the child's illness*, Treating the child, Appendix: "When transfer is not possible".

III. ASSESS AND CLASSIFY the baby for birth weight

ASK:

- are there any problems with low birth weight or weight gain?
- Does the child have feeding difficulties?
- Is the child ? If yes, please specify, how many times in 24 hours?
- the child usually receive other foods or drinks? If so, how often?
- How do you feed your baby?

OBSERVE, LISTEN, FEEL:

- Determine birth weight (if less than 7 days)
- Determine weight for age. Observe mother's nipples for abnormalities (short, umbilicated nipples).

Sort birth weight

Weight ≤ 1500 g	VERY SMALL BABY	➤ Ensuring warmth is very important during evacuation ➤ Evacuate the baby to hospital
Weights between 1500 g and 2500 g	LITTLE BABY	➤ Keeping the Nné warm ➤ Encourage mothers to breastfeed every 2 to 3 hours ➤ Evaluate the newborn's condition every day ➤ Do not let him out until he feeds properly or gains weight and body temperature remains stable
Weight ≥ 2500 g	NORMAL BIRTH WEIGHT	Advice feeding the baby (breast-feeding)

IV. Then look for jaundice.

ASK

When did jaundice first appear?

OBSERVE

- Look for a yellow color (jaundice) in the eyes and skin.
- Are the infant's palms and soles yellow?

Classifying icterus

• Jaundice in less than 24 hours old - Or • Yellow palms and soles in infants all ages	ICTERE GRAVE	➤ <i>Treatment to avoid hypoglycemia</i> ➤ <i>Emergency transfer to hospital</i> ➤ <i>Teaching the mother how to keep the baby warm during transfer</i>
• Jaundice after 24 hours of life - AND • Non-yellow palms and soles	ICTERE	➤ Explain to the mother how to administer care at home ➤ Advise mother to return immediately if palms and turn yellow ➤ If the infant is more than 3 weeks old, transfer him to hospital for a check-up. ➤ See you in a day
• No jaundice	NO ICTERE	➤ Advising mothers on home care for young infants

V. Does your baby have diarrhea?

IF SO, OBSERVE AND FEEL :

- Examine the infant's general condition:
-Is your baby restless and irritable?
- Infant movements :
-Do infants move only when stimulated, then stop moving?
it not move at all?
- Look for sunken eyes.
-Pinch the skin on abdomen. Does the skin fold disappear?
-very slowly (more than 2 seconds)?
-or slowly?

What is diarrhea in infants?

An infant has diarrhea if there is a change in the usual appearance of the stool, which has become more numerous and more liquid (containing more water than feces).

Frequent, semi-solid stools in breast-fed infants are not diarrhea.

Classify DIARRHEA FOR DEHYDRATION

Signs	Classify	Treatment (Pre-transfer treatment in bold)
Two of the following signs: • Moves only when stimulated or not at all • Sunken eyes • Skin fade very slowly	DEHYDRATION SEVERE	➤ If the infant no other serious classification: -Provide liquids for dehydration. (Plan C) OR ➤ If the infant also has another serious classification: -Refer URGENTLY to hospital, with mother giving frequent sips of ORS on the way. -Advise mother to continue breastfeeding. ☐ Tell mother how to keep infant warm during transfer to hospital.
Two of the following signs: • Restless, irritable • Sunken eyes • Skin fade slowly	DEHYDRATION Slight	➤ Give fluids and breast milk for mild dehydration (Plan B). OR ➤ If the infant also has another serious classification: -Refer URGENTLY to hospital, with mother giving frequent sips of ORS on the way. Advise mother to continue breastfeeding. ➤ Tell mother when to return immediately. ➤ Follow-up in 2 days if no improvement.
• Not enough signs to classify severe or mild	NO DEHYDRATION	➤ Give fluids and breast milk to treat diarrhea at home (Plan A). ➤ Tell the mother when to come back immediately. ➤ Follow-up in 2 days if improvement

VI. Then look for HIV infection in infants

ASK • Has the mother had an HIV test? If yes : – Is the serological test POSITIVE or NEGATIVE? • Has the baby had an HIV test? If yes : – Is the virological test POSITIVE or NEGATIVE? – Is the serological test POSITIVE or NEGATIVE? If no : – par - Test not performed on mother or infant. If the mother's HIV test is positive and there is NO positive virological test in infants, ASK : – Is the baby breastfed now? – Was the infant breastfeeding at the time of testing or before testing? – Is the mother on treatment and the infant on antiretroviral prophylaxis?	Classify INFECTION HIV based on test results			➤ Refer or start the mother on anti-retroviral treatment if she is not already on it. ➤ Advise mother on home care. ➤ Ensure regular monitoring in line with national guidelines. ➤ See the baby again in 14 days
		Positive serological test in infants or • Positive HIV test in mother AND negative virological test in infant who is , or who has not breastfeeding for less than 6 se- hands. or • Positive HIV test in mother, and the infant has not yet been tested	HIV EXPOSURE :POSSIBLE HIV INFECTION	➤ Give cotrimoxazole for prophylaxis from 4-6 weeks of age. ➤ Start or continue anti-retroviral prophylaxis according to risk assessment. ➤ Perform a virological test on the infant. ➤ Refer or start the mother on antiretroviral treatment if she is not already on it. ➤ Advise mother on home care. ➤ Regular monitoring in accordance with national guidelines ➤ See the baby again in 14 days
		• HIV test not performed mother or in infants	HIV INFECTION: STATUS IN- KNOWN	➤ Initiate HIV testing and counseling. ➤ Perform an HIV test on the mother and, if positive, perform a virological test on the infant. ➤ Test the infant's serology first, if the mother is not available. If positive, have a virological test
		Negative HIV test in mother or negative virological test in infant	LOW-PROBABILITY HIV INFECTION	➤ Treat, advise and follow up on ongoing infections. ➤ Advising mothers on diet and their own health

VII. THEN LOOK FOR FEEDING OR LOW WEIGHT AGE IN BREAST-FED INFANTS.

Request	Observe
- Does the feeder suckle? if so how many times in 24 hours? - Does the nouris- son receive other food or liquids? - If so, how times? - What do you feed it? on?	- Determine weight for age. - Weight < 2 kg? - Weight for age < -2 Z score - Look for ulcerations or white patches in the mouth (oral candidiasis)
ASSESSING BREASTFEEDING : - Has the baby suckled in the last hour? If the infant has not suckled in previous hour, ask the mother to put the baby to the breast and observe breastfeeding for 4 minutes. (If the infant has suckled in previous hour, ask the mother if she is breastfeeding. can wait and tell you when the baby wants to nurse). - Is the baby latching on well? <i>Good latch Poor latch No latch at all</i>	
TO EVALUATE THE , SEE IF : - the areola is more visible above the upper lip than below the lower lip - the mouth is wide open - the lower lip is splayed - chin touches breast	
Does the infant suckle efficiently (i.e., with slow, steady sucks)? deep suction sometimes interspersed with pauses)? <i>Effective suction Not effective suction No suction at all</i> ☐ Cleanse a blocked nose that could interfere with feeding.	

Classifying
ALIMENTA-
TION

SIGNS	CLASSIFY	PROCESS
Weight < 2 kg in infants aged less than 7 days	VERY LOW WEIGHT FOR AGE	➤ REFER hospital for Kangaroo care ➤ Treat to prevent hypoglycemia. ➤ Advise mother to keep infant warm during transfer
- Poor latch or - Suction no efficient, or - Less than 8 feeds in 24 hours, or - Receives other foods or liquids, or - Weight < -2 Z score, or - Oral candidiasis (ulcerations or white patches in the mouth)	POWER SUPPLY PROBLEM and/or LOW WEIGHT AGE	➤ If the latch is not good or if sucking is not effective, teach the mother the correct position and . ➤ If less than 8 nipples in 24 hours, advise mother increase nipple frequency, breastfeed as often and for as long as the baby wants, day and night. ➤ If the baby is given other foods or liquids, advise the mother to milk more, reduce other foods and use a cup. ➤ If he doesn't suckle at : - Refer for breastfeeding advice and relactation if possible - Learn how to prepare breast-milk substitutes correctly and how to use a cup. ➤ Teaching the mother how to feed and keep warm a low-weight baby at home ➤ If oral candidiasis is present, teach the mother to treat it at home. ➤ Advise mothers on how to care for their infants at home. ➤ Follow-up for BUCCAL CANDIDOSIS or PROBLEM FOOD in 2 days. ➤ Follow-up for WEIGHT LOW FOR AGE in 14 jou
Weight ≥ -2 Z score and other signs of inappropriate feeding.	NO FEEDING PROBLEMS	➤ Advise mothers on how to care for their infants at home. ➤ Praise the mother for feeding her baby well.

VIII. THEN LOOK FOR FEEDING OR LOW WEIGHT IN NON-BREASTFED INFANTS

If an infant has no indications justifying emergency transfer to hospital

ASK	OBSERVE AND FEEL Determine :	Classify diet and weight	- P/T < - 3ET - Weight 2kg <	SEVERE MALNUTRITION	- Teaching the mother to keep the infant warm during transfer - hypoglycemia - Transferring the newborn hospital
What kind of milk do you give? How many times a day and night? How much do you give each time? How do you prepare the milk? Ask the mother to demonstrate and explain how she prepares the alim-ents and gives it to the infant. Do you give breast milk? What food and liquid do you give in addition to the substitute food? How do you give milk (cup or bottle)? How do you clean your baby's feeding utensils?	Weight for height, weight for age Height for age Weight under 2kg? P/T < - 2ET Look for ulcerations or white patches in the mouth (thrush) Observe the mother's nipples for abnormalities (<i>umbilicated, short nipples, etc.</i>).		- Poorly or unhygienically prepared milk or - Gives incorrect substitute foods or - HIV-positive mother mixing breast milk with other foods before 6 months of age or - Is bottle-fed or <u>P/T < - 2 ET</u> <u>Or</u> - Thrush (ulcerations or white patches in the mouth). <u>or</u> - Mouth malformations	FEEDING AND/OR PONDERAL INSUFFICIENCY	- Food advice - Explain the guidelines for safe alternative food - Identify the mother's and family's food-related concerns. - If the mother uses a bottle, show her how to use a cup - In the presence of candidiasis, show the mother how to treat at home - Review for FOOD PROBLEM or CANDIDOSIS in 2 days - See you again in 7 days for PONDERATIVE INSUFFICIENCY FOR AGE - In the event of thrush, teach the mother to treat it at home. - Teach mothers to care for their infants at home. - Review any feeding or lily of the valley after 2 days. Review all problem of low weight age after 14 days
			Not underweight and no other signs inadequate power supply	NO FEEDING OR WEIGHT PROBLEMS	- Learn how to care for infants at home. - Congratulate the mother on feeding her baby well Infant.

IX. NEXT, CHECK INFANT'S IMMUNIZATION STATUS

Vaccination schedule

AGE	VACCINES
Birth	BCG+ Polio 0+ Hep b0
6 weeks	Penta 1+ Pcv 13, 1+ Polio 1+ Rotavirus1

- ⇒ *Give any doses missed this visit.*
- ⇒ *Vaccinate sick children unless they are transferred.*
- ⇒ *Tell the mother or accompanying person when to return for the next dose.*

X. CHECKING MOTHER'S STATE OF HEALTH

Administer a dose of 200,000 IU of vitamin A within 6 weeks of delivery

XI. ASSESSING OTHER PROBLEMS